IN THE DISTRICT COURT OF APPEAL

FIFTH DISTRICT OF FLORIDA

Case No. 5D2026-2448

TOM TREVISANI, M.D. a/k/a THOMAS P. TREVISANI, and CELEBRITIES CHOICE, INC.,
Appellants,

v.

VERONICA GONZALEZ HERNANDEZ,
Appellee.

ON APPEAL FROM THE 5TH CIRCUIT IN AND FOR LAKE COUNTY, FLORIDA, CASE NO. 35-2024-CA-001996-AXXX-01

APPELLANTS' INITIAL BRIEF

HICKS, PORTER & STEIN, P.A.
5301 Blue Lagoon Drive, Suite 900
Miami, FL 33126
Tel: (305) 374-8171
dstein@mhickslaw.com
pgonzalez@mhickslaw.com
eclerk@mhickslaw.com
Appellate Counsel for Appellants

MCEWAN, MARTINEZ, DUKES, HALL & VANCOL, P.A.
108 East Central Blvd., Orlando, FL 32801
Tel: (407) 423-8571
jengel@mmdorl.com
NOS@mmdorl.com
Counsel for Appellants

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INTRODUCTION

Appellants, TOM TREVISANI, M.D., and CELEBRITIES CHOICE, INC., Counter-Defendants below, seek reversal of the trial court's order denying their motion to dismiss Plaintiff's medical malpractice claims for failure to comply with the presuit statutes. As will be shown, Counter-Plaintiff, VERONICA GONZALEZ HERNANDEZ, failed to corroborate her medical malpractice claims with an affidavit of an expert who specializes in the same plastic surgery specialty as Dr. Trevisani, as required by Chapter 766 of the Florida Statutes. As a result, the trial court was required to dismiss Ms. Gonzalez's claim.

BASIS FOR JURISDICTION

This Court has jurisdiction pursuant to Florida Rule of Appellate Procedure 9.130(a)(3)(H). Appellant Dr. Trevisani moved to dismiss Appellee Ms. Gonzalez's medical malpractice claim on the ground that the qualifications of Ms. Gonzalez's corroborating expert witness under section 766.102(5), Florida Statutes, are insufficient to satisfy the conditions precedent for filing her medical malpractice claims. Dr. Trevisani timely appealed the non-final order denying his motion to dismiss.

STATEMENT OF THE CASE AND FACTS

Dr. Trevisani is a board certified plastic surgeon. (A.6, ¶7; A.100, 327-28, 348). In September of 2023, Ms. Gonzalez underwent a rhinoplasty procedure with Dr. Trevisani to cosmetically improve her nose. (A.66, ¶10). Ms. Gonzalez was not happy with the result and took to the Internet to express her displeasure with Dr. Trevisani. (A.5, ¶5]). In October of 2024, Dr. Trevisani filed a complaint and amended complaint against Ms. Gonzalez asserting a claim for defamation. (A.5-11, 12-33). Ms. Gonzalez then filed a counterclaim against Dr. Trevisani, and a third-party complaint against his facility, Celebrities Choice, asserting claims for medical malpractice arising out of alleged negligence in his performance of the rhinoplasty. (A.54-100).

Dr. Trevisani and Celebrities Choice (collectively "Dr. Trevisani") moved to dismiss Ms. Gonzalez's medical malpractice claims on several grounds, including her failure to comply with the mandatory presuit requirements set forth under Chapter 766. (A.101-13). Dr. Trevisani contended that Ms. Gonzalez failed to corroborate her claims with an affidavit from an expert in the same plastic surgery specialty as Dr. Trevisani, let alone in the preceding

three years before the date of the alleged negligence, as required under sections 766.102(5), 766.202(6), and 766.203(2), Fla. Stat. (A.108-10).

In support of the motion to dismiss, Dr. Trevisani contended that Ms. Gonzalez's presuit expert, Kim Murray, M.D., who performed a revision surgery on Ms. Gonzalez's nose, did not include a curriculum vitae with his presuit affidavit. (A.110). Dr. Murray only attested that he was board certified in head and neck surgery by the American Academy of Otolaryngology. (A.88). In contrast, he attested that he was merely "highly experienced in cosmetic primary and revision rhinoplasty," and a member of the American Academy of Facial Plastic and Reconstructive Surgery and the American Rhinologic Society. (*Id.*). Dr. Murray's own medical records for Ms. Gonzalez's revision surgery, which were attached to her pleading, were on letterhead for "South Florida ENT Associates." (A.96).

In response to the motion to dismiss, Ms. Gonzalez still did not provide Dr. Murray's curriculum vitae, did not submit an updated corroborating affidavit, and did not request an evidentiary hearing to allow her to demonstrate that Dr. Murray specializes in plastic

surgery. Rather, Ms. Gonzalez contended her countercomplaint should not be dismissed because Dr. Murray's affidavit indicated he was "a licensed Florida physician, board certified in Head and Neck Surgery, affiliated with facial plastic and rhinologic specialty societies," had performed the revision surgery on Ms. Gonzalez, and was "actively engaged" in the relevant surgical field. (A.128).

Following a hearing, on May 7, 2026, the trial court issued an order finding that Ms. Gonzalez "failed to comply with the mandatory Pre-Suit Requirements of Florida Statute section 766," and dismissed the countercomplaint and third-party complaint without prejudice. (A.173). Ms. Gonzalez moved for reconsideration on the grounds, as relevant to this appeal, that the court made no finding of prejudice to Dr. Trevisani, did not "follow the liberal standard of the presuit investigation process," and failed to make evidentiary findings of fact "as required by section 766.102(2)(b)"—a provision that only holds the statute does not apply to claims for lack of informed consent. (A.175-88). Ms. Gonzalez's motion was denied. (A.251).

Despite the trial court's dismissal of Ms. Gonzalez's medical malpractice claim based on her "fail[ure] to comply with the

mandatory Pre-suit Requirements of Florida Statute section 766," Ms. Gonzalez did not obtain a supplemental affidavit from Dr. Murray or make any attempt to reinitiate (i.e., comply with) presuit. Rather, eleven days after the court dismissed the malpractice claim, Ms. Gonzalez filed an "amended counterclaim and third-party complaint" attaching the same presuit materials attached to the original complaint. (A.189-250).

The primary difference from the dismissed pleading was Ms. Gonzalez's attachment to the amended pleading of a second notice of intent that Ms. Gonzalez had served on Dr. Trevisani on November 20, 2025, to correct a service error with the first notice. (A.229-30). The November 2025 re-notice letter had made clear that it was enclosing the same affidavit from Dr. Murray that was provided with the July 22, 2025, notice of intent. (*Id.*).

Dr. Trevisani moved to dismiss the amended complaint based on the court's prior ruling, and Ms. Gonzalez's continued failure to obtain a compliant corroborating affidavit. (A.252-61). Dr. Trevisani contended, again, that Ms. Gonzalez had wholly failed to corroborate her malpractice claims in presuit with an expert who specialized in the same specialty as Dr. Trevisani in the three years

preceding the alleged negligence. (*Id.*) In her written response, Ms. Gonzalez only contended that Dr. Murray's affidavit was sufficient to show the claim was not frivolous because he is licensed to practice medicine in Florida, board certified in Head and Neck Surgery by the American Academy of Otolaryngology, "highly experienced in cosmetic primary and revision rhinoplasty after having performed 1200 cases," and treated Ms. Gonzalez. (A.272-73).

Dr. Trevisani filed a reply in which he "reassert[ed] the arguments and authority previously made" in the prior filings directed at Ms. Gonzalez's noncompliant presuit, that even after dismissal she had failed to comply with the mandatory screening requirements of Chapter 766, and that the medical malpractice claim should be dismissed with prejudice. (A.286-92, 288).

A hearing was held on Dr. Trevisani's second motion to dismiss on July 8, 2026. (A.293). Dr. Trevisani's counsel reminded the court that it had already dismissed the malpractice claim based on Ms. Gonzalez's failure to comply with Chapter 766 presuit, and that "[n]othing that was refiled as an amended counterclaim or

complaint changes the fact that the presuit compliance was not met months and months ago." (A.295-96).

As it related to Dr. Murray's qualifications, Ms. Gonzalez again argued Dr. Murray was qualified to corroborate claims against Dr. Trevisani because he is licensed to practice medicine in Florida, board certified in Head and Neck Surgery by the American Academy of Otolaryngology, a member of professional societies for plastic surgery, and "highly experienced" in cosmetic, primary and revision rhinoplasty. (A.301).

The court asked Ms. Gonzalez's counsel: "[H]ow is your argument here today different than your argument you made on May 5, 2026?" (A.309). In response, Ms. Gonzalez's counsel only stated that the amended counterclaim also attached the November 20, 2025 re-notice letter that she had sent to correct a service error with the original notice of intent—the same letter that enclosed all of the original presuit materials from July of 2025, including Dr. Murray's original affidavit. (A.309-10; *see also* A.150-51).

On July 16, 2026, the court issued an order denying Dr. Trevisani's motion to dismiss and inexplicably finding that the amended counterclaim "satisfies the requirements of Florida Statute

766." (A.314-15). Dr. Trevisani then filed a motion for clarification and reconsideration which pointed out that, in addition to Ms. Gonzalez's failure to demonstrate that her presuit expert practiced in the same specialty as Dr. Trevisani, Florida Department of Health records affirmatively established that Dr. Murray has no certification in plastic surgery, rendering him unqualified to corroborate Ms. Gonzalez's claims. (A.327-29, 343-45).

Dr. Trevisani timely filed the instant appeal from the order denying the motion to dismiss. The trial court thereafter denied Dr. Trevisani's motion for clarification and reconsideration without explanation. (A.350).

SUMMARY OF THE ARGUMENT

As Ms. Gonzalez wholly failed to submit any record evidence demonstrating that her presuit expert Dr. Murray is qualified to corroborate her medical malpractice claim against Dr. Trevisani, dismissal of her claim was required. The record, including Ms. Gonzalez's presuit materials, only shows that Dr. Murray specializes in head and neck surgery and is board certified in otolaryngology. Dr. Trevisani, in contrast, is a board certified plastic surgeon.

Chapter 766 and the case law interpreting it is clear: the statutory requirement that a plaintiff corroborate her claims with an expert who specializes in the same specialty means what it says. Practicing in similar specialties, or treating the same body part, is insufficient under sections 766.102(5) and 766.206. Ms. Gonzalez had every opportunity to submit record evidence showing that Dr. Murray specializes in plastic surgery, but failed to do so. This failure mandated dismissal.

None of Ms. Gonzalez's arguments advanced below save her claim. Neither the statute nor any decisional law allow a court to consider prejudice to a defendant as a result of the failure to comply with the presuit statutes, as such prejudice is presumed. The fact that Dr. Murray also performs plastic surgeries is a fact that the courts have held does not create a "same specialty" under the statute. Dr. Murray's alleged membership in professional organizations for plastic surgery does not equate to the expertise that is required for a physician to qualify as a specialist in the same specialty.

Ms. Gonzalez's presuit materials and arguments below also wholly failed to demonstrate that Dr. Murray devoted professional

time to Dr. Trevisani's specialty during the three years preceding Dr. Trevisani's care and treatment of Ms. Gonzalez on September 5, 2023. This provides additional grounds for reversal.

As the undisputed record establishes that Dr. Murray was not qualified to corroborate the claim against Dr. Trevisani, this Court should reverse and remand for dismissal of that claim.

STANDARD OF REVIEW

An appellate court's review of an order granting or denying a motion to dismiss based on a determination of the sufficiency of a presuit affidavit is reviewed de novo. *See Davis v. Karr*, 264 So. 3d 279, 281 (Fla. 5th DCA 2019); *Holden v. Bober*, 39 So. 3d 396, 401 (Fla. 2d DCA 2010).

ARGUMENT

I. THE ORDER DENYING DR. TREVISANI'S MOTION TO DISMISS MS. GONZALEZ'S MEDICAL MALPRACTICE CLAIMS MUST BE REVERSED.

Ms. Gonzalez plainly did not comply with the mandatory presuit provisions of Chapter 766, and the trial court's inexplicable failure to dismiss her claim was legally erroneous and must be reversed. A board certified otolaryngologist is not statutorily qualified to render opinions against a board certified plastic

surgeon, regardless of whether they perform the same surgeries or treat the same patients. Reversal is required pursuant to Florida Statutes sections 766.202(6) and 766.102(5).

A. Chapter 766 expert corroboration requirements

Chapter 766 sets forth an extensive presuit investigation procedure that both the claimant and prospective defendant must follow before a medical negligence claim may be brought or maintained in court. *See Kukral v. Mekras*, 679 So. 2d 278, 280 (Fla. 1996).

Before filing a medical malpractice claim, a prospective plaintiff is required to determine whether reasonable grounds exist to believe that someone acted negligently and caused his or her injury. *Id.* "This requires both investigation and corroboration of the results of the investigation by 'a verified written medical expert opinion'." *Largie v. Gregorian*, 913 So. 2d 635, 638 (Fla. 3d DCA 2005); *see also* § 766.104(1), Fla. Stat. (no medical action may be filed "unless the attorney filing the action has made a reasonable investigation" to determine there are "grounds for a good faith belief that there has been negligence.").

There are two key components to the presuit expert corroboration requirement. First, pursuant to section 766.203(2), the expert affidavit must corroborate claimant's contention that the named defendant or its agents were negligent in their care and treatment of the claimant and that such negligence resulted in injury to the claimant. *See Largie*, 913 So. 2d at 637-40; *Rell v. McCulla*, 101 So. 3d 878, 879 (Fla. 2d DCA 2012).

Second, pursuant to section 766.202(6), the plaintiff's medical expert must be statutorily qualified under a specific provision of section 766.102 to render an opinion regarding the alleged negligent conduct. § 766.202(6), Fla. Stat. (the corroborating medical expert must "mee[t] the requirements of an expert witness as set forth in s.766.102.").

Where a prospective defendant physician is a specialist, section 766.102(5)(a)1 requires that the claimant's corroborating expert "[s]pecialize in the same specialty" as the prospective defendant.

Upon a challenge to a presuit expert's qualification, a trial court is required to review the nonmovant's presuit affidavit to determine whether it meets the requirements of Chapter 766,

including whether the expert is qualified under sections 766.202 and 766.102, Fla. Stat. § 766.206(1) & (2), Fla. Stat. If the court determines that the plaintiff filed a medical malpractice claim without providing "a verified written medical by an expert witness as defined in s. 766.202," "the court shall dismiss the claim." § 766.206(2), Fla. Stat. *See also Potparic v. Barnes*, 419 So. 3d 1100, 1104 (Fla. 4th DCA 2025) (if a plaintiff's corroborating affidavit is found to be facially deficient, "section 766.206(2) commands" that the court shall dismiss the claim).

"These presuit requirements are designed to 'alleviate the high cost of medical negligence claims through early determination and prompt resolution of claims'." *Holden v. Bober*, 39 So. 3d 396, 400 (Fla. 2d DCA 2010), citing *Weinstock v. Groth*, 629 So. 2d 835, 838 (Fla. 1993).

B. Dr. Murray is not in the Same Specialty as Dr. Trevisani, Requiring Dismissal.

Under the plain language of section 766.102(5)(a)1, Dr. Murray —a board certified otolaryngologist—is not in the "same specialty" as Dr. Trevisani—a board certified plastic surgeon. The alleged facts that Dr. Murray is "highly experienced" in plastic surgery, is a member of plastic surgery societies, and performed a revision

surgery on Ms. Gonzalez are wholly insufficient to render him a "specialist" in plastic surgery under section 766.102(5)(a)1.

Although "same specialty" is not defined, Florida's appellate courts have held that the phrase "is to be taken literally and is not synonymous with physicians in different specialties providing similar treatment to different areas of the body." *Blackshear v. Haynes*, 425 So. 3d 798, 802 (Fla. 2d DCA 2025), citing *Riggenbach v. Rhodes*, 267 So. 3d 551, 555 (Fla. 2d DCA 2019), *disapproved on other grounds*.

Physicians who specialize in the same "branch of medicine or surgery" are in the "same specialty." *Pradaxay v. Kendrick*, 387 So. 3d 437, 440 (Fla. 6th DCA 2024). This can generally be ascertained by comparing the physicians' board certifications. *E.g.*, *id.* at 440 (holding defendant and presuit expert "specialize in the same branch of medicine, as illustrated by their identical OB-GYN board certifications").

It is manifest that plastic surgery—which focuses on the remodeling, repair, or restoration of the appearance and function of body parts—and otolaryngology—which deals with the diagnosis and treatment of diseases of the ear, nose and throat—are not the

"same specialty."[1] *See Myers v. Pasco Cty. Sch. Bd.*, 246 So. 3d 1278, 1279 (Fla. 1st DCA 2018) (orthopedic surgeons and neurosurgeons are not in the "same specialty" even though both treat back injuries: "A physician who provides similar services in a different specialty does not qualify as a doctor in the "same specialty" because—quite simply—"same" is *different* than "similar."); *Riggenbach*, 267 So. 3d 551 (expert who specialized in plastic surgery and otolaryngology was not in the same specialty as defendant orthopedic surgeon, requiring dismissal of complaint); *Blackshear*, 425 So. 3d 798 (nephrologist was not in same specialty as vascular surgeon); *Davis v. Karr*, 264 So. 3d 279, 280 (Fla. 5th DCA 2019) (presuit affidavits from emergency room physician, radiologist and nurse were insufficient to corroborate claim against orthopedic surgeon); *Clare v. Lynch*, 220 So. 3d 1258 (Fla. 2d DCA 2017) (podiatrist was not in same specialty as orthopedic surgeon), *disapproved on other grounds*; *Edwards v. Sunrise Ophthalmology ASC, LLC*, 134 So. 3d 1056 (Fla. 4th DCA 2013) (under former version of statute requiring "similar" specialties, infectious disease

[1]These definitions are derived from the American Heritage Dictionary—the same source referenced by the Courts in *Pradaxay* and *Ghamra v. Williams*, 423 So. 3d 479 (Fla. 2d DCA 2024), and found at www.AHDictionary.com.

doctor was not qualified to corroborate claim against ophthalmologist whom plaintiff contended was responsible for her infection).

Ms. Gonzalez's argument below that Dr. Murray is qualified because he is "highly experienced" with cosmetic surgeries and performed Ms. Gonzalez's revision surgery is contrary to the plain language of section 766.102(5)(a)(1) and the case law interpreting this provision. *See Riggenbach*, 267 So. 3d 551.

In *Riggenbach*, the plaintiff alleged her orthopedic surgeon negligently performed wrist surgery. The plaintiff's presuit expert, a plastic surgeon and otolaryngologist, testified he had performed the same type of surgery that was alleged to have been negligently performed on the plaintiff multiple times, and described his practice as including plastic reconstructive and hand surgery. 267 So. 3d at 553. Explaining that "same specialty" is "not synonymous with physicians with different specialties providing similar treatment to the same area of the body," the *Riggenbach* Court rejected the trial court's finding that the physicians were in the same specialty and ordered that the complaint be dismissed. *Id.* at 555.

The *Clare* Court similarly held in ordering dismissal of a malpractice complaint: "While both doctors' [orthopedic surgeon and podiatrist] practice focus primarily on foot and ankle surgery, these two doctors have different training and practice in different specialties." *Id.* at 1260-61.

Here, the fact that Dr. Trevisani and Dr. Murray may both perform cosmetic surgeries is similarly insufficient. The physicians' differing board certifications demonstrates that they too "have different training and practice in different specialties." *Id.*

Nor is it relevant under section 766.102(5) that Dr. Murray belongs to a plastic surgery academy and society. As the Second District explained in *Ghamra*, a "specialist" is "[o]ne who has developed professional *expertise* in a particular specialty or subject area." 423 So. 3d at 482 (emphasis added). Membership in a professional society or organization does not equate to expertise, and does not render the physician a specialist in that branch of medicine.

Finally, there is no record dispute below as to Dr. Murray's lack of qualification. Ms. Gonzalez correctly alleged that Dr. Trevisani is a plastic surgeon and Celebrities Choice is a "plastic

surgery practice." (A. [CC p. 13). Dr. Murray could have, but did not, attest that he specializes in plastic surgery. Ms. Gonzalez did not provide Dr. Murray's curriculum vitae with her notice of intent, and his affidavit only stated that he is a board certified otolaryngologist. (A.81-93). After Dr. Trevisani challenged Dr. Murray's qualification by pointing out that he had failed to state in the presuit materials that he practiced in the same specialty as Dr. Trevisani (A.107-12), Ms. Gonzalez did not contend in response or try to establish that Dr. Murray specialized in the field of plastic surgery. Rather, she merely contended that Dr. Murray "is *engaged* in the profession of plastic surgery." (A.127-28) (emphasis added). Being "*engaged*" in a particular profession is not synonymous with specialization.

In addition, Ms. Gonzalez: did not state in her motion for reconsideration of the court's May 2026 dismissal order that the trial court misconstrued Dr. Murray's specialization; did not request an evidentiary hearing; and did not submit any evidence that Dr. Murray's specialization was plastic surgery, despite multiple opportunities. Rather, Ms. Gonzalez only argued, incorrectly, that her expert's presuit affidavit "substantially complied" with the presuit statutes. (A.179-82).

Ms. Gonzalez's response in opposition to the motion to dismiss her amended counterclaim fared no better. (A.272-73). The response only reiterated that Mr. Murray is "engaged in" the professional of plastic surgery for the same insufficient reasons as previously stated. At the July 8, 2026 hearing, Ms. Gonzalez's counsel merely reiterated these arguments, and incorrectly suggested that the attachment of the November 20, 2025, re-notice of intent somehow cured things. (A.301, 309-10). It did not, as that re-notice expressly enclosed the same insufficient corroborating affidavit included with the original notice of intent. (A.150-64).

Thus, Ms. Gonzalez did not even create a fact issue that could have potentially required an evidentiary hearing.

There is also no merit to Ms. Gonzalez's contention below that the court was first required to find that her noncompliance prejudiced Dr. Trevisani in order to dismiss the complaint. The decision on which she relied, *Morris v. Muniz*, 252 So. 3d 1143 (Fla. 2018), addressed the requirement to find prejudice when the movant is asserting the other side's failure to comply with the requirements of informal presuit discovery. *Id.* at 1157-61.

In contrast to *Morris*, this case does not involve an alleged noncompliance with presuit discovery, but a failure to comply with a threshold requirement of presuit that the potential claim was corroborated by a qualified expert. The courts have made it abundantly clear prejudice is presumed when a claimant fails to properly corroborate a medical malpractice claim, as it deprives the defendant of a statutory right which causes irreparable harm. *See Rell v. McCulla*, 101 So. 3d 878, 880-81 (Fla. 2d DCA 2012) ("Certainly, where a plaintiff in a medical malpractice action has failed to satisfy the presuit notice requirements set forth in section 766.203(2), the defendant in such an action would suffer a material injury that could not be remedied in a postjudgment appeal if the action was allowed to proceed."); *Shands Jacksonville Med. Ctr., Inc. v. Pusha*, 254 So. 3d 1076, 1080 (Fla. 1st DCA 2018) (irreparable harm to medical malpractice defendant is presumed when plaintiff is erroneously excused from providing presuit corroboration).

Dr. Trevisani was not required to make any additional showing in regard to the prejudice he has endured and continues to endure as a result of being made to defend a claim that was not subject to the rigors of Chapter 766. Reversal is required.

II. THE ORDER ADDITIONALLY MUST BE REVERSED BECAUSE MS. GONZALEZ FAILED TO ESTABLISH THAT DR. MURRAY SATISFIES THE THREE-YEAR ACTIVE-PRACTICE REQUIREMENT OF SECTION 766.102(5)(a)2.

Section 766.102(5)(a) imposes two separate, conjunctive qualification requirements on any expert who would corroborate a claim against a specialist. The first, addressed above, is that the expert "[s]pecialize in the same specialty" as the defendant. The second, set forth in subparagraph 2, is that the expert have devoted professional time to that same specialty during the three years immediately preceding the date of the alleged negligence. Nothing in Ms. Gonzalez's presuit materials, or anywhere else in the record, establishes Dr. Murray satisfies this second requirement. Dr. Trevisani raised this precise deficiency below, and Ms. Gonzalez never cured it. (A.110, 257). Dismissal was therefore mandatory under section 766.206(2).

A. Section 766.102(5)(a)2 imposes an independent, time-specific qualification requirement.

A presuit corroborating opinion must come from a "medical expert as defined in s. 766.202(6)." § 766.203(2), Fla. Stat. Section 766.202(6), in turn, defines a "medical expert" as one who, among other things, "meets the requirements of an expert witness as set forth in s. 766.102." § 766.202(6), Fla. Stat.; *see Morris*, 252 So. 3d

at 1152 (section 766.202(6) "refers to and incorporates the requirements of section 766.102"); *Baptist Med. Ctr. of the Beaches, Inc. v. Rhodin*, 40 So. 3d 112, 118 (Fla. 1st DCA 2010) (presuit "medical expert" must be duly and regularly engaged in practice, hold a health care professional degree, and "meet the requirements of an expert witness addressed in section 766.102"); *Hunt v. Huppman*, 28 So. 3d 989, 992 (Fla. 2d DCA 2010) (presuit expert must satisfy each component of the section 766.202(6) definition, including the requirements of section 766.102). Section 766.102(5) provides in relevant part:

> A person may not give expert testimony concerning the prevailing professional standard of care unless the person is a health care provider who holds an active and valid license and conducts a complete review of the pertinent medical records and meets the following criteria:
>
> (a) If the health care provider against whom or on whose behalf the testimony is offered is a specialist, the expert witness must:
>
> 1. *Specialize in the same specialty* as the health care provider against whom or on whose behalf the testimony is offered; and
>
> 2. Have devoted professional time during *the 3 years immediately preceding the date of the occurrence that is the basis for the action* to:
>
> a. *The active clinical practice of, or consulting with respect to, the same specialty*;

> b. Instruction of students in an accredited health professional school or accredited residency or clinical research program in *the same specialty*; or
>
> c. A clinical research program that is affiliated with an accredited health professional school or accredited residency or clinical research program in *the same specialty*.
>
> § 766.102(5)(a), Fla. Stat. (emphasis added).

The statute's text makes three things plain. First, subparagraphs 1 and 2 are joined by the conjunctive "*and*": an expert must satisfy both and satisfying one does not excuse the other. Second, subparagraph 2 is anchored to a fixed period of "*the 3 years immediately preceding the date of the occurrence that is the basis for the action*." This three-year period is not fixed to the expert's career generally, and not to the date the expert signs the affidavit, but instead to the three years immediately preceding the date of occurrence that is the basis for the action. Third, each of the three alternative means of satisfying subparagraph 2 - clinical practice or consulting, teaching, or clinical research - must be devoted to "*the same specialty*." Accordingly, even an expert who was clinically practicing during the relevant period does not qualify unless that professional time was devoted to the defendant's specialty. *See Riggenbach*, 267 So. 3d at 553 - 55 (quoting both

subparagraphs and holding expert unqualified notwithstanding his testimony that he had performed the type of surgery at issue several times during the previous three years, because he did not practice in the defendant's specialty).

The Florida Supreme Court's decision in *Morris* illustrates how the three-year requirement operates. There, the defendants challenged the presuit expert's qualifications under section 766.102(5)(a)2. *Morris*, 252 So. 3d at 1147-48. The expert, a board certified obstetrician-gynecologist, swore in her affidavit that she "was engaged in full-time patient care until March 2008," and the alleged negligence occurred in January 2009. *Id.* at 1146-47, 1156. The Court held the expert qualified because her unrefuted affidavit placed her full-time clinical practice in the defendant's specialty within the three years immediately preceding the occurrence. *Id.* at 1156-57 (applying the 2011 version of the statute, under which subparagraph 2.a. referred to "the same or similar specialty"). In so holding, the Court reaffirmed that it is the "more specific time period" in section 766.102 (the period preceding the occurrence) that defines when the expert must have been engaged in practice, not the expert's activities at the time the opinion is rendered. *Id.* at

1153-54 (quoting *Fort Walton Beach Med. Ctr., Inc. v. Dingler*, 697 So. 2d 575, 580 (Fla. 1st DCA 1997)). *Morris* thus confirms that the look-back period is measured from the date of the occurrence, and that it is satisfied by facts showing the expert's practice in the relevant specialty during that period. *See also Rhodin*, 40 So. 3d at 117, 120 (applying the parallel three-year requirement of section 766.102(5)(c); presuit expert qualified where her affidavit described her practice "during the past three years" and her curriculum vitae documented the dates of her teaching and consulting work).

Whether an expert's stated qualifications satisfy the statute is a question of law reviewed de novo where, as here, the facts concerning the expert's background are unrefuted. *Morris*, 252 So. 3d at 1155-56 ("Determining whether a presuit expert witness is qualified under the statute involves reviewing the expert's stated qualifications and comparing those with what is required under the statute. The trial court is in no better position than the appellate court to conduct this analysis."); *Apostolico v. Orlando Reg'l Health Care Sys., Inc.*, 871 So. 2d 283, 286 (Fla. 5th DCA 2004); *Rhodin*, 40 So. 3d at 116. In conducting that review, courts "must review the sufficiency of [the plaintiff's expert's] qualifications and his

corroborating affidavit to determine if it complies with the statutory requirements of chapter 766." *Holden*, 39 So. 3d at 400 (quoted with approval in *Morris*, 252 So. 3d at 1155).

B. Nothing in the presuit materials shows that Dr. Murray devoted professional time to Dr. Trevisani's specialty during the three years preceding September 5, 2023.

The occurrence that is the basis for Ms. Gonzalez's claim is the rhinoplasty Dr. Trevisani performed on or about September 5, 2023. (A.191, ¶¶ 10, 70; A.207). The relevant period under section 766.102(5)(a)2 therefore ran from September 5, 2020 through September 5, 2023. Dr. Murray's affidavit is silent as to that period. The entirety of what it says about his background is that he is "a licensed medical doctor in Florida, in good standing with the Florida Department of Health, having practiced since 2006"; that he is "board certified in Head and Neck Surgery by the American Academy of Otolaryngology"; that he is "a member of the American Academy of Facial Plastic and Reconstructive Surgery and the American Rhinologic Society"; and that he is "highly experienced in cosmetic primary and revision rhinoplasty after having performed over 1200 cases." (A.214, ¶¶ 2-5).

None of these statements addresses what Dr. Murray was doing professionally between September 2020 and September 2023. The statement that he has "practiced since 2006" does not say what he practiced, where, in what specialty, or to what extent during the relevant period. A license in good standing satisfies only the threshold licensure language in the introductory clause of section 766.102(5); it says nothing about the separate requirements of subparagraph (a)2. Board certification and professional-society membership are credentials, not "professional time" devoted to anything during a defined period. And the claim of "over 1200" rhinoplasty cases is untethered to any timeframe, and is otherwise irrelevant for purposes of complying with section 766.102(5). The affidavit does not say whether any of those cases were performed in the three years before the occurrence rather than at some point over a nearly two-decade career. Nor does the affidavit claim that Dr. Murray instructed students in, or participated in a clinical research program in, plastic surgery during the relevant period. *See* § 766.102(5)(a)2.b.-c., Fla. Stat.

Moreover, to the extent the presuit materials reveal anything about the nature of Dr. Murray's practice, they point away from

compliance. The only specialty credential he identifies is in otolaryngology. (A.214, ¶ 3). Further, the documents that accompanied his affidavit, which amount to the only other evidence of his practice in the presuit materials, are on the letterhead of "South Florida ENT Associates," and his operative report bears a "South FL. ENT Assoc." transmittal header on every page. (A.220-26). Thus, even if the affidavit could be read to suggest that Dr. Murray was clinically active between 2020 and 2023, nothing in the record suggests that his professional time was devoted to "the same specialty" as Dr. Trevisani as subparagraph 2 requires.

No curriculum vitae or other document was ever provided to fill these gaps. The notice of intent's list of enclosures identifies only a medical authorization, Dr. Murray's affidavit, and "[c]opies of medical records rel[ied] upon by Dr. Murray in signing the affidavit." (A.209). Ms. Gonzalez did not supply a curriculum vitae or a supplemental affidavit in response to the motion to dismiss, despite Dr. Trevisani's express objection that none had been provided. (A.257, ¶ 23; A.272-73). This record stands in stark contrast to *Morris*, where the expert's affidavit placed her full-time practice in the defendant's specialty through a date within the look-back

period, 252 So. 3d at 1156, and *Rhodin*, where the affidavit expressly addressed the expert's practice "during the past three years" and her curriculum vitae documented the dates of her relevant work, 40 So. 3d at 117.

Nor does *Morris* suggest that Dr. Murray's affidavit suffices. The Court there rejected the trial court's demand that the expert's affidavit "state specifically" the level of practice in which she engaged during the three-year period, explaining that "the statutes do not demand such specificity." *Morris*, 252 So. 3d at 1157. But Dr. Trevisani's objection is not that Dr. Murray's affidavit lacks detail about the level of his practice during the look-back period; it is that the affidavit says nothing at all about practice in the defendant's specialty during that period. The *Morris* expert swore to full-time patient care, as a board certified physician in the defendant's specialty, through a date within the three-year window. *Id.* at 1156. Dr. Murray's affidavit, by contrast, identifies no practice in plastic surgery at any time, let alone during the three years preceding September 5, 2023. Under *Morris*, an affidavit whose unrefuted statements establish each statutory requirement suffices; nothing in *Morris* excuses an affidavit that is silent as to one of them.

It was Ms. Gonzalez's obligation to furnish corroboration from a qualified expert. Section 766.203(2) places on the claimant the duty to provide corroboration "by the claimant's submission of a verified written medical expert opinion from a medical expert as defined in s. 766.202(6)," and that definition incorporates the requirements of section 766.102. §§ 766.202(6), 766.203(2), Fla. Stat. On this record, there is no basis on which the trial court could have found that Dr. Murray met the requirements of section 766.102(5)(a)2.

C. Dr. Murray's post-occurrence treatment of Ms. Gonzalez cannot satisfy the statute.

Below, Ms. Gonzalez's principal response to the challenge to Dr. Murray's qualifications was that he is "the very physician who performed revision surgery on Ms. Gonzalez," having consulted with her in July 2024 and operated on her in August 2024. (A.272-73, 301). She argued that Dr. Murray therefore was not "some detached, non-practicing expert" and that his "connection to practice" was not "abstract or stale." (A.272-73). That argument responds, at most, to the separate "duly and regularly engaged" component of section 766.202(6). *See Rhodin*, 40 So. 3d at 118; *Hunt*, 28 So. 3d at 992 (treating the section 766.202(6)

requirements and the additional requirements of section 766.102 as independent prerequisites). It does not address section 766.102(5)(a)2 at all. Indeed, under *Morris*, even the "duly and regularly engaged" requirement is measured by reference to the time period preceding the occurrence, not by the expert's activities after it. 252 So. 3d at 1153 - 54.

Dr. Murray's consultation and revision surgery occurred in July and August 2024, ten to eleven months after the September 5, 2023 occurrence and thus entirely outside the statutory window. (A.238-40, ¶¶ 6, 9]). The statute looks backward from "the date of the occurrence that is the basis for the action"; professional activity after that date is, by the statute's plain terms, irrelevant to subparagraph 2. *See Morris*, 252 So. 3d at 1153-54 (the expert's role is to opine on "the professional standard of care existing at the time of the occurrence that is the basis for action," and the time period in section 766.102 defines when the expert must have been engaged in practice). And in any event, a subsequent treating physician's care of the claimant does not convert his specialty into the defendant's. Whatever Dr. Murray's familiarity with Ms. Gonzalez's condition, the Legislature has determined that only an

expert who devoted professional time to the defendant's specialty in the three years before the occurrence may corroborate a claim against a specialist. The principle that Chapter 766 is to be construed liberally in favor of access to the courts, *see Kukral*, 679 So. 2d at 284, does not permit a court to disregard the statute's express qualification requirements.

D. Dr. Trevisani raised this deficiency below, and Ms. Gonzalez never cured it.

Dr. Trevisani specifically raised this deficiency in his motion to dismiss the amended counterclaim and third-party complaint, arguing that Dr. Murray's affidavit "provides no information . . . regarding his practice of medicine – including in which discipline or specialty - within the preceding 3 years immediately preceding the date of the occurrence that is the basis of the action," citing section 766.102(5), and that "no curriculum vitae accompanied Dr. Murray's affidavit." (A.257, ¶ 23). He quoted the "active clinical practice of, or consulting with respect to, the same specialty" language of section 766.102(5)(a)2.a. and argued that the affidavit "fails to meet the requirements of §766.102." (A.257-58, ¶¶ 26-29). He reasserted those arguments in his reply (A.288, ¶ 12), and at the hearing counsel pointed to "the lack of a C.V. . . . to prove that the

credentials of the . . . purported expert under the statute." (A.310-11).

Ms. Gonzalez never responded to this argument on its terms. Her response did not identify any professional activity by Dr. Murray between September 2020 and September 2023, did not supply a curriculum vitae, and did not offer a supplemental affidavit. Instead, she relied entirely on the existing affidavit and Dr. Murray's 2024 treatment of her. (A.272-73). At the hearing, counsel again relied solely on the affidavit's recitation of Dr. Murray's license, board certification, society memberships, case volume, and 2024 treatment. (A.301). The amended pleading simply re-attached the same affidavit, executed July 22, 2025, that accompanied the original notice of intent. (A.207-19).

Because the record contains no facts establishing that Dr. Murray satisfies section 766.102(5)(a)2, he is not a "medical expert" as defined in section 766.202(6), and his affidavit cannot supply the corroboration section 766.203(2) requires. Where the claimant fails to provide "a verified written medical expert opinion by an expert witness as defined in s. 766.202," "the court shall dismiss the claim." § 766.206(2), Fla. Stat.; *Potparic*, 419 So. 3d at 1104. The

trial court's failure to do so was error, and the order should be reversed on this independent ground as well.

CONCLUSION

WHEREFORE, Appellants, TOM TREVISANI, M.D., and CELEBRITIES CHOICE, INC., respectfully request that this Court reverse the order denying their motion to dismiss the countercomplaint and third-party complaint asserting claims for medical malpractice.

Respectfully submitted,

By: /s/ Dinah S. Stein
DINAH S. STEIN
Florida Bar No.: 98272
5301 Blue Lagoon Drive, 9th Floor
Miami, FL 33126
Tel: (305) 374-8171
dstein@mhickslaw.com
pgonzalez@mhickslaw.com
eclerk@mhickslaw.com
Appellate Counsel for Appellants

-and-

JOSEPH W. ENGEL, JR.
Florida Bar No. 91046
MCEWAN, MARTINEZ, DUKES,
HALL & VANCOL, P.A.
108 E. Central Boulevard
Orlando, FL 32801
Tel: (407) 423-8571
jengel@mmdorl.com
NOS@mmdorl.com
Counsel for Appellants

CERTIFICATE OF SERVICE

WE HEREBY CERTIFY that on this 8th day of October, 2026, the foregoing was filed using Florida's E-Filing Portal and a copy served via the E-Portal to:

Cynthia Conlin, Esq.
Cynthia Conlin, P.A.
1824 E. Robinson Street, Orlando, FL 32803-5935
attorney@conlinpa.com
cynthia@conlinpa.com
service@conlinpa.com
Counsel for Appellee

By: /s/ Dinah S. Stein
DINAH S. STEIN
Florida Bar No.: 98272

CERTIFICATE OF COMPLIANCE

The foregoing complies with the font and word count requirements of Fla. R. App. P. 9.045 & 9.210. It is typed in Bookman Old Style, 14-point font and does not exceed 13,000 words.

By: /s/ Dinah S. Stein
DINAH S. STEIN
Florida Bar No.: 98272